**Breast adjuvant treatment only · NCCP 00588h.** These materials cover olaparib after treatment for high-risk early breast cancer. Other cancers use different eligibility, duration and follow-up; use the matching NCCP indication and your treating team's plan.

NCCP 00588 · VERSION 5B

# Olaparib · adjuvant breast treatment

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Reference timing, not booked appointments. Ask who requests each test, when it is due, who reviews the result and whom to contact if it is missing. Delays can change cycle numbers; time-based monitoring remains time-based.

| Check | Before starting | First cycle / month | Early treatment | Ongoing treatment | Change / completion |
|---|---|---|---|---|---|
| **Blood count, kidneys and liver** | Baseline FBC, renal and liver profile; recover previous blood toxicity before starting | First repeat around four weeks; earlier if symptoms or results require | Every four weeks | Every four weeks through the twelve-month course | Recheck abnormalities; severe/ persistent blood toxicity may need marrow investigation |
| **Symptoms, medicines and tablet supply** | Review previous treatment, medicines, supplements and anti-sickness plan | Check nausea, intake, fatigue and adherence; do not wait if unwell | Review at blood/clinic contacts; check new medicines | Continue reviews and arrange supplies | Review recovery and the final olaparib stop plan |
| **Surgery, radiation and hormone treatment** | Complete the agreed local treatment; confirm timing after radiation | Olaparib begins after local-treatment recovery; hormone treatment may overlap | Continue separately prescribed endocrine therapy where indicated | Endocrine therapy can continue alongside olaparib | Olaparib finishes by the planned year; endocrine treatment may continue |
| **Breast surveillance and scans** | Confirm surgical follow-up and breast-imaging plan | No automatic olaparib-specific CT scan | Breast follow-up and symptom-directed investigation | Follow the agreed breast surveillance plan; no generic three-month CT rule | Continue surveillance after olaparib; investigate new symptoms promptly |
| **Echo, ECG and bone density** | Check if previous/other treatments or existing conditions require testing | No routine olaparib-specific echo or ECG | Keep any separate endocrine or cardiac monitoring plan | DXA follows endocrine treatment/fracture risk, not the olaparib calendar | Continue only as indicated by the relevant treatment and risk plan |
| **Pregnancy and contraception** | Pregnancy exclusion/test where relevant; contraception discussion | Follow agreed contraception | Consider repeat testing where indicated | Continue contraception throughout | Women: six months after; men/ partners: three months. No breastfeeding for one month after |
| **Breast surveillance** | Confirm surgery / remaining breast tissue | Agree first mammography date | New symptoms need assessment | Annual mammography of remaining breast tissue | No routine CT / tumour markers if asymptomatic |

THE WHOLE TREATMENT JOURNEY

# Olaparib · adjuvant breast treatment: the whole pathway

### Germline BRCA1/2-associated, HER2-negative high-risk early breast cancer

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First cycle / month | Early treatment | Ongoing treatment | Change / completion |
|---|---|---|---|---|---|
| **Treatment** NCCP | Confirm exact indication and biomarkers | Usually 300 mg twice daily; follow the individual prescription | Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course. | Continue with safety / benefit review | For breast indication 00588h: up to one year, or earlier if the cancer recurs or side effects make treatment unsuitable. This is different from olaparib durations in ovarian, prostate and pancreatic cancer. |
| **Surgery / chemotherapy** NCCP · SMPC | Neoadjuvant chemo → surgery, or surgery → adjuvant chemo | Recover from prior treatment | Olaparib after completion of chemotherapy | Review lingering treatment effects | Ongoing survivorship care |
| **Radiotherapy** NCCP · SMPC | Complete indicated local treatment, including RT | Allow the agreed recovery interval before olaparib | Do not infer concurrent RT from this map | No ongoing RT unless separately indicated | Review local effects |
| **Endocrine therapy if HR+** NCCP · SMPC | Confirm HR status / prescribed endocrine plan | May overlap adjuvant olaparib | Continue prescribed endocrine treatment | Endocrine duration differs from olaparib | May continue after olaparib finishes |
| **Safety checks** NCCP · SMPC | Baseline bloods and individual risks | Early checks from the monitoring table | Regimen-specific checks continue | Review results before continuation | Resolve abnormalities / agree next plan |
| **Scans / follow-up** ESMO_EBC2024 | Confirm baseline and treatment setting | Agree the next imaging milestone | Use the matching surveillance / response plan | Review benefit and symptoms | Continue setting-specific follow-up |

# What each check is for

### 1 Blood tests

Confirm the germline BRCA result and breast eligibility. Review blood count, kidney and liver tests, recovery from previous treatment, medicines and supplements, and pregnancy testing where relevant. Confirm the surgery/radiotherapy sequence and the planned start. NCCP specifies FBC, kidney and liver tests every four weeks for the first twelve months. Reviews also assess nausea, appetite, energy, bruising, infections and breathing symptoms. Extra tests or an interruption may be needed if results or symptoms change.

Sources: NCCP · SMPC

### 2 Heart checks

Olaparib alone has no routine ECG or echo surveillance timetable in this NCCP regimen. Heart checks may still be needed because of previous anthracycline/HER2 treatment, existing disease or new symptoms.

Sources: NCCP · SMPC

### 3 Imaging

After early breast cancer treatment, annual mammography is generally used for remaining breast tissue: both breasts after conservation, or the opposite breast after unilateral mastectomy. Routine mammography of the mastectomy side is not required. Routine CT, bone scans and tumour markers are not recommended solely to look for recurrence in an asymptomatic person. Safety blood tests for ongoing olaparib are a separate requirement. New symptoms need assessment between appointments.

Sources: ESMO_EBC2024

### 4 Bone health

There is no olaparib-specific DXA schedule. If an aromatase inhibitor or ovarian suppression is also prescribed, retain that treatment’s separate bone-density and bone-health plan.

Sources: NCCP · SMPC

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.